

The Psychiatric Bible - A comprehensive review on the DSM and its ability to diagnose

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Abstract:

The DSM and its definition based system was created in 1970 with the emergence of the DSM 3rd edition. Researchers Robins and Guze utilized an assumption they had about schizophrenia, and mental illnesses as a whole. This was that mental illnesses were defined by the symptoms that patients displayed. By recording the common set of symptoms that were seen in a clinical setting, Robins and Guze believed they were able to categorically define an illness through a list of symptoms. This initial study was used as the backbone of the DSM 3rd edition in its ability to create a common set of guidelines for a diagnosis for mental illness. This categorical approach is utilized in the DSM 4th edition, and the DSM 5th edition. 21st century findings have adopted a newer dimensional approach, in opposition to the categorical approach. Research from National Institute of Mental Health (NIMH) and the National Institute of Health (NIH) have shown mental illnesses to fall on a dimensional spectrum, with coexisting symptoms and simultaneous illnesses, that involve severity as a measure. I am arguing for the repurposing of the DSM in both standardized education, and higher-level education. Additionally, I am calling for the concept of the approaches of mental illness (dimensional and categorical) and supplementary items such as the RDoC matrix, to enter introductory psychology and psychiatry courses.

The Psychiatric Bible

The DSM is a standard in American mental health as most doctors, students, and clinical professionals follow it for diagnosis advice without question. However, they will miss that the definitions within it fail to address a patient's dynamic sociocultural context, and favor a vague sense of checkbox - like symptoms. Because of this shortcoming of the DSM, along with a lack in the psychological and psychiatric educational system to push students to understand this caveat, mental health is harder to fight. Additionally incorrect diagnoses are given, and wrong treatments are prescribed. We must educate professionals at the clinical level about how the clarity of DSM definitions work, and adapt the education system to understand a deeper level of diagnosis, rather than a one size fits all style like the DSM

The DSM - I had a controversial history. Heavily based on the concept of “the mad,” which stemmed from the witch hunts of the 17th century, the original DSM was loosely based on “science.” Those with “the mad ” were taken to the “Cuckoo’s Nest,” referring to a psychiatric ward for schizophrenics, where inmates were often treated as animals and were subject to miracle serum testing. Psychoanalytic introspection and theory was the result of this era, and greatly dominated this manual and the DSM - II, where homosexuals were granted normalcy thanks to the American Psychiatric Association. This theory made way for the “blame the mother” theory, where women were found guilty of producing a subpar upbringing to their kin, resulting in autism, or any other misunderstood illness. With mother facing such scrutiny, many doctors began to prescribe her “little helper” better known as Diazepam. Alternatively a minor tranquilizer, these first antipsychotic medications were backed by doctors across America who wanted to help “bored housewives get through their busy day.” Since women decided they did not want to quit their wartime jobs, or accept their husband’s advances, this psychoanalysis -

backed drug was pushed by medical professionals as a hope for a “cure.” As over 35 million prescriptions were delivered by 1957, psychoanalysts had believed that this new cure for women would help solve infidelity, and their desire to leave the home.

With the arrival of the Civil Rights Era, the roots for feminism in America were planted and brought it to the public eye at levels it had never seen before. Psychiatricians could not get away with simply blaming women anymore. Additionally, the post-war era showed Americans how the Soviets blurred the lines of mental illness to forcibly outcast and medicate their “ill.” And the final nail in the coffin for psychoanalytic theory was psychologist David Rosenhan, and his study “On Being Sane in Insane Places.” Also referred to as the Rosenhan Experiment, eight false patients (including Rosenhan) who had no history of mental illness, arrived at psychiatric hospitals in an attempt to test the validity of psychiatry as a whole. 7 of the patients were diagnosed with schizophrenia, and the last handed a manic-depressive label. The pseudo-patients had an average stay of 19 days in the hospitals, and led to the mass questioning of psychiatry as a whole. With an entire industry under such fire, the American Psychiatric Association decided to begin work on an entirely new manual, one based in “science.”

Robert Spitzer of Columbia University began pushing the American Psychiatric Association to develop a new manual in response to the embarrassment psychiatry was facing. Work began for the DSM - III and its definition based system in 1974, with an ultimate release date in 1980. The DSM - III created a landmark in psychiatric and psychological studies: the categorical system for diagnosis. Much of the groundwork for this new system was mirrored from the American Journal of Psychiatry volume 126, issue 7; More specifically, a study from Robins and Guze regarding Schizophrenia. These academics aimed to create an applicable method for determining diagnostic validity. They proposed five phases: clinical description,

laboratory study, exclusion of other disorders, follow-up study, and family study. This was the first true validation model created for the aim of identifying a mental illness, and was heavily referenced in the creation of the DSM - III. However an abbreviated version of this model was utilized in the creation of the DSM - III, with a heavy focus on the first three stages of the model proposed by Robins and Guze. In practice on patients with schizophrenia, the last two stages of the model, follow-up study and family study, highlighted that poor prognosis cases were to be separated from good prognosis cases. This was because, according to Robins and Guze, a good prognosis meant an entirely different disease, rather than mild schizophrenia. This idea was eventually extrapolated to the DSM with the motif in mind of defining a new illness when one diagnosis did not fit. The lack of the usage of a spectrum - based system was transferred into the DSM - III, and the DSM - IV and V subsequently. The very core of a categorical - based system stems from this study from Robins and Guze.

As the DSM - III and IV were continuously “strengthened,” an increasing number of definitions were added following the structure set by Robins and Guze. A new definition for each illness that did not fit the last set of criteria was amended, creating a robust set of textual criteria. Supporting the criteria came the illness name, where the symptoms simultaneously played the role as criteria for diagnosis. Here is where Robins and Guzes’ assumption regarding Schizophrenia began to shape the DSM, and psychiatry, in a holistic way. By providing these lists of criteria and diagnoses a name yields them as an entity of their own. This is known as the issue of reification. English Philosopher John Stuart Mill supported this ideology by explaining how humans will naturally believe that “ whatever receives a name must be an entity or thing, having an independent existence of its own.” Regardless of what the American Psychiatric Association may assert through the DSM and its definition-based system, this is what will occur.

The issue ultimately lies in the fact that the causation of most mental illnesses is not truly known when utilizing the DSM. Depression is not the actual entity that is causing a patient to experience certain emotions, but the DSM will treat it like the opposite. We do not know what is causing your mental hardships, so we assign you “X Term.” “X Term” is the culprit, but now what? Maybe take [*purchase*] “Y drug?” This is explicitly apparent in the DSM through its classification for ADHD. The DSM will outline ADHD as an illness that “affects educational and occupational performance.” However, suggesting that ADHD causes compromised performance in these areas creates a reified definition, as these symptoms are also the criteria for diagnosis. Ultimately because of this core issue in the DSM, we see mental illnesses often being used as a scapegoat for an answer. Rather than holding a medical answer, the DSM can now be seen as more of a cultural currency. It serves as an entity that has a “promise of validating individuals' pain and legitimizing their identities, but often fails to protect individuals from delegitimation and stigma.”

As we reached the publishing of the fifth edition of the DSM in 2013, reification was something the American Psychiatric Association, and much of the academics in psychiatry were aware of. However the solution was not something as simple as lexicon choice or the rewording of definitions, but rather an entirely new paradigm was needed to interpret mental illnesses. But with such research only coming to light towards the end of the first decade of the twenty-first century, creating an entirely new system for classifying diagnoses would be unrealistic as the DSM was set to release in 2013. Hyman’s “The Diagnosis of Mental Disorders: The Problem of Reification (2010)” very clearly identified the errors with the DSM. Additionally, the United States Government and the National Institute of Mental Health began work on the Research

Domain Criteria in 2010, known as the RDoC, a project with the purpose to counter the DSM with dimensional definitions rather than categorical ones.

The DSM - V did ultimately release in 2013, still bearing categorical definitions. However the APA did understand their shortcomings, and now stated that the DSM - V does not “constitute comprehensive definitions of underlying disorders.” The APA recognizes that mental illnesses are “cognitive, emotional, behavioral, and physiological processes that are far more complex than can be described in brief summaries.” The definitions outlined in the DSM are not to be utilized and the symptoms are not to be “checked - off” during the creation of a diagnosis without “careful clinical history” and a summary of the “social, psychological, and biological factors” of the patient in regard.

While the APA’s update regarding their definitions may “solve” the issue of reification, it seems the real issue was forgotten: the patients. All hope is not lost however, as much progress has taken place on the NIMH’s RDoC. The RDoC utilizes objective scientific evidence via a dimensional approach on mental health. Rather than checking off symptoms, like the DSM, patients are able to have their severity of symptoms taken into account. A 2017 publishing from the University of Toronto’s clinical science department titled “Six Years of Research on the National Institute of Mental Health’s Research Domain Criteria (RDoC) Initiative: A Systematic Review” found that “The Research Domain Criteria (RDoC) initiative has successfully integrated genetics, brain circuits, and physiology research findings with behavior and self-reports in studying the biology of mental illness.” With such findings taking place, it is time for students to become educated about the dimensional approach as well. One of the most popular high school courses for psychology, AP Psychology mentions nothing of the dimensional approach in their “AP Psychology Course and Exam Description, Effective Fall

2024.” However “5 Instructional Periods” are to be dedicated to the “Selection of Categories of Psychological Disorders.” Additionally, the DSM is referenced as a manual used to “classify mental disorders” by the College Board, but there is zero mention of the RDoC and dimensional manuals.

There is no doubt that progress has been made in the psychiatric and psychological industries. It was just 60 years ago when “science” determined that women were the issue. Now in 2024, and “science” once again has created another issue, this one being reification. In order to discover more, there will always be mistakes, but “science” must keep pushing forward, or humanity would still be holding trials for the “witches” with mental illnesses. Rather than find animosity in the jargon that dominates, it is our job as humanity to remember why it is we started researching in the first place.

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